جوستاف روسي انترناشيونال - مصر (EPE) *

طلب علاج أول مرة / متردد

إسم المريض رباعياً : مرفت عبدالقادر امين امام
المحافظة : الجيزه **الحي / المدينة :** العياط **العنوان :** ممم
رقم البطاقة : 27604112100321 **رقم التليفون :** 01094051121 **تاريخ تسجيل الإستمارة :** 23-10-2026
العمر(تاريخ الميلاد) : 11-04-1976 **المهنة :** ربه منزل

الإقرار

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل:-التأمين الصحي-القوات المسلحه-الشرطة-الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك

الإخصائي الاجتماعي **المقر بما فيه**
التوقيع:(----------) **التوقيع:**(----------)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)
نتائج الأبحاث : (مرفق)
التشخيص النهائي : سرطان بالثدي (مبادرة)
خطة العلاج : أبحاث أورام مسح ذري علي العظام [12966] **قسم العلاج :** الاورام
التعليق على التشخيص : ----------

تقرير اللجنة الثلاثية المتخصصة

الاسم: د/محمد كيلاني **الوظيفة:** ---------- **التوقيع:** ----------
الاسم: د/ندى حسن **الوظيفة:** ---------- **التوقيع:** ----------
الاسم: د/نرمين رمضان **الوظيفة:** ---------- **التوقيع:** ----------

أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية.
كما تقر اللجنة بأن البيانات المدونة فى هذا النموذج صحيحة.

تعتمد صحة البيانات المرفقة
رئيس اللجنة و مدير المستشفي.

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفي قبل الصرف.

أعضاء اللجنة

ا.د/ ---------- **ا.د/** ---------- **د/** ---------- **د/** ----------

قرار اللجنة

التشخيص النهائي : ----------
خطة العلاج : ----------
المستشفي : ----------**قسم :** ----------
جهة العلاج : ----------**نفقات :** ----------
ملاحظات : ----------

رئيس	مدير عام	ختم
المجلس الطبي المتخصص	مديرية الشئون الصحية	شعار الجمهورية

Name: مرفت عبدالقادر امين امام
National ID: 27604112100321

تقرير طبي

السيد

تحية طيبة وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بأن نوضح فيما يلي تقريرا عن حالته

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026

وبعد الكشف والفحوصات والمتابعة وجد انة

المريضة تعاني من ورم غير حميد بالثدى و تحتاج الي Bone scintigraphy

التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين

Patient's Name:	**MERVAT ABD AL KADER AMEIN EMAM**
Date:	**2026-04-06**
Referred by:	Prof.Dr.
Hospital:	**Goustave Roussey International Egypt**

MULTISCLICE POST CONTRAST C.T. EXAMINATION OF THE CHEST, ABDOMEN AND PELVIS REVEALED:

A known case of managed left breast cancer, compared to previous study dated 12/11/2025, the current study reveled

I- CHEST:

- Left MRM showed clear operative bed with no evidence of enhanced lesions or fluid collection.
- Still seen left internal mammary and superior mediastinal prominent lymph nodes
- Sill seen right axillary lymph nodes, the largest measures about 18x12mm for further sono-mammography
- **Progressive course** of the few left upper lobar pulmonary nodules, the largest measures about 19x13 mm at current study compared to 10x9mm a the previous study .
- Left upper lobar subpleural reticulations likely post-radiation sequel
- Few pre-vascular lymph node enlargement, the largest measures about 19x10 mm.
- No pleural or pericardial collection.
- Mild enlarged cardiac size

II- ABDOMEN AND PELVIS:

- The liver is mildly enlarged in size showing homogenous parenchymal attenuation. Still seen ill defined lesion at segment IVb measures 15x11mm with newly developed lesion at segment VIII measures 10mm
- No dilated biliary radicles.
- The gall bladder is well distended, average wall thickness with no stones.
- Both kidneys are normal site, average sized and showing smooth regular cortical outline and preserved parenchymal thickness and function. No stones or back pressure changes.
- Normal C.T appearance of the spleen, pancreas, suprarenal glands, aorta and IVC.

CT Scan
[[DOCTYPE:CT_SCAN]]

Patient's Name:	**MERVAT ABD AL KADER AMEIN EMAM**
Date:	**2026-04-06**
Referred by:	Prof.Dr.
Hospital:	**Goustave Roussey International Egypt**

- The urinary bladder shows no evidence of gross masses or diverticulae or stones.
- Normal appearance of the pelvic organs with no pelvic masses or collections.
- Subcentimeteric para-aortic and inguinal lymphadenopathy, the largest measures about 12mm in left inguinal region.
- No ascites.
- **The examined skeleton showed : stationary course**
  - No lytic or sclerotic lesions.
  - D12 burst fracture with retropulsion over spinal cord.

Opinion:

- **Progressive course regarding the size of the left upper lobe pulmonary nodules**
- **Newly developed small hepatic focal lesion for dynamic MRI study**
- **stationary rest of study**

Dr.Nourhan Ramzy

Dr.Nourhan Ramzy

CT Scan
[[DOCTYPE:CT_SCAN]]

Name: مرفت عبدالقادر امين	
MR. : 4129	

Detailed Medical Report

السيد	

تحية طيبة وبعد,,

إيماء للطلب المقدم من		**بشأن تقرير طبي للمريض**	
بقسم:		**رقم طبي:**	

ننشرف بأن نوضح فيما يلي تقريرا عن حالته

تاريخ وساعة الدخول :		**تاريخ وساعة الخروج :**	

وبعد الكشف والفحوصات والمتابعة وجد انه

The patient is a 50-year-old individual with no history of diabetes mellitus or hypertension. The clinical course began with a first presentation of metastatic breast cancer to the liver in November 2021, for which systemic therapy with Herceptin, Taxol, and carboplatin was initiated. A left total mastectomy was performed in March 2022, followed by adjuvant trastuzumab (TDM) administered once weekly for 12 cycles, concluding in December 2022, after which the patient was maintained on follow-up until August 2024. Disease progression was noted in the lung, prompting further Herceptin IV and Taxol until June 2025, after which therapy was switched to Tykerb plus Xeloda, continued until August 2026.

Imaging demonstrated stable disease of a right axillary lymph node measuring 1.7 × 1.2 cm, a progressive left upper lobar pulmonary mass measuring 3.2 × 2.1 cm, multiple mediastinal lymph nodes up to 3 × 1.9 cm, and a decreasing right hilar focal lesion now 0.8 cm. A new biopsy has been obtained for reassessment of estrogen receptor, progesterone receptor, and HER2 status.

The referring team recommended consideration of EnHurt from an NGO and initiation of gemcitabine plus carboplatin. The multidisciplinary team agreed to pursue EnHurt from the NGO and to start Tykerb plus Navilbine, with the new biopsy results to guide further management.

تحريرا في ,

التاريخ :	23-09-2026	**الوقت :**	

الطبيب المعالج :

يعتمد ,





National ID · Front & Back
[[DOCTYPE:ID_FRONT_BACK]]

جوستاف روسي

مبادرة رئيس الجمهورية لدعم صحة المرأة المصرية

الست المصرية هي صحة مصر

MDT FORM 8 / 12 / 2025 G

Name	أمنت عبدالقادر	MRN	3191/2024
		ID	CV٦٠٤١١ C١٠٣C١
Age	49	Phone	01094051124
Menopausal status	Post ?	Marital status	
NRS SCORE	PS 0-1	Family history	-ve
BMI	↑	REASON OF MDT	Mixed Response to tx
CO morbidity	-ve		

Clinical Data

BX: IDC

ER:	PR:	HER 2:	Ki67:
0/8	0/8	+3	

R — NBC — Bone L

Lung

LN3O

Sonomammography:

3/2024 → Lt T_{4b} N_3M_0 NA (TCHP) → Lt MPH ($_{yp}T_1S N_{4/1}$)

by Kadcyla (11/2023) TNM

Mets Workup:

7/2024 → Lt Supraclav biopsy → IDC HR-ve Her2 +ve 3

→ Taxol / Herceptin (6cycl) → RD

Recommendations:

more than 3m of herceptin kept on (Tykerb / Xeloda) (Since 5/2025)

11/2025 → DP Lt lung nodule (6mm → 10x9mm)

RD

RD LN3. bone.

Recommended to keep on Tykerb / Xeloda if approved by MDT (3m & reassess) 11/25

MDT OUTCOME & PLAN

Kinase Progression, Tykerb / Xeloda continue for 3 more then CT chest / Ab / pelvis (close f-u)

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric
M. Kelly		Nermeen 8/12/2025			

MDT.com
[[DOCTYPE:MDT_FORM]]

Patient Name	: الأستاذه / ميرفت عبدالقادر امين	Request No.	: 13004892
Patient No.	: 27604112100321	Request Date	: 21/07/2024
Path No.	: 2528/24	Verification Date	: 12/08/2024
Sex	: Female	Financial Category	: اورام دار السلام - مبادره
Age	: 48 Y	Department	: Out Patient
Referreing Doc.	:	IHC No.	:
		Registered by	: Mrs.Mayada Hegazy

IMMUNOHISTOCHEMISTRY REPORT

Clinical Data: Known case of left breast cancer (treated) with pathologically enlarged left supra-clavicular lymph node.

Nature of Specimen: Tru-cut biopsy, collected at 21/7/2024.

Technique:
Sections were prepared from the paraffin block, processed in Bench Mark ULTRA machine (Ventana) and stained by rabbit monoclonal antibodies using ultra view HRP DAB as chromogen and hematoxylin as counter stain. The sections treated against:
ER clone (1E2), PR clone (SP1), Her2 clone (4B5), & Ki67 clone (30-9).

Results:
IPT (Positive control showed appropriate reactivity):
ER: No nuclear reaction in the tumor cells (score 0 /8).
PR: No nuclear reaction in the tumor cells (score 0 /8).
Her2: Strong complete membrane reaction in >10% of tumor cells (score 3+).
Ki67: 35% nuclear reaction of tumor cells.

Conclusion:
Known case of left breast cancer (treated) with pathologically enlarged left supra-clavicular lymph node, Tru-cut biopsy:
Invasive duct carcinoma, Grade III.
IPT:
- ER: Negative (score 0 /8).
- PR: Negative (score 0 /8).
- Her2: Positive (score 3+).
- Ki67: Positive in 35% of tumor cells.

Verified by :
DR.Menna Nabil

غرفة رقم (4) – الدور الأول – مبنى رقم (987) – كورنيش النيل – مصر القديمة – القاهرة – مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

Immunohistochemistry
[[DOCTYPE:IHC]]

atient Name	الأستاذه / ميرفت عبدالقادر امين	Request No.	13004892
'atient No.	27604112100321	Request Date	21/07/2024
ath No.	2528/24	Verification Date	12/08/2024
Sex	Female	Financial Category	اورام دار السلام - مبادره
Age	48 Y	Department	Out Patient
Referreing Doc.		Registered by	Mrs.Mayada Hegazy

IMMUNOHISTOCHEMISTRY REPORT

Scoring system: "ER, PR"
ASCO-CAP ER/PR Test Guideline Recommendations: proportion of cells staining (0, no staining; 1, -1%; 2, between 1% and 10%; 3, between 11% and 33%; 4, between 34% and 66% and 5, between 67%-100% of the cells staining). Intensity of positive tumor cells (0, none; 1, week, 2, intermediate; and 3, strong). Total score from 0 to 8.

Scoring system: "Her2"
Reference: ASCO-CAP Her2 Test Guideline Recommendations, 2013.IHC 0: as defined by no staining observed or membrane staining that is incomplete and is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 1+: as defined by incomplete membrane staining that is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 2+: based on circumferential membrane staining that is incomplete and/or weak/moderate and within < 10%of the invasive tumor cells or complete and circumferential membrane staining that is intense and within < 10% of the invasive tumor cells. IHC 3+: based on circumferential membrane staining that is incomplete, intense < 10% of tumor cells.

Signature
Prof. Dr. Lubna Omar.
Dr. Menna Nabil.

Verified by :
DR.Menna Nabil

غرفة رقم (4) – الدور الأول – مبنى رقم (987) – كورنيش النيل – مصر القديمة – القاهرة .. مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

Immunohistochemistry
[[DOCTYPE:IHC]]

National Women's Health Initiative
Pathology Lab
معمل باثولوجي المبادرة القومية لصحة المرأة

Pathology Report

Name: مرفت عبد القادر امين امام Age: 48 years.
Clinical data: left breast mass
Specimen type: Tru-cut biopsy
Date (Received): 28/7/2021. Date(reported): 7/8/2021
Lab. Number: 1581/ 21 Hospital Number: مبادره

Gross Examination:

- Procedure: tru cut biopsy (four cores up to 1.5 cm long, totally submitted)
- Localization: Lt
- Immunohistochemistry technique: Sections prepared from paraffin blocks for immune-staining using Bench-Mark ULTRA system for ER, PR, Her-2 and Ki67
  ER clone (1E2), PR clone (SP1), Her-2 clone (4B5), & Anti-K-67 (30-9)

Microscopic examination:

- Histological type: IDC
- Grade II
- Additional pathologic findings: calcification

Diagnosis:

Lt breast mass, tru-cut biopsy:

- Invasive duct carcinoma GII
- Biomarkers:
  - ER: negative nuclear reaction (Allred score 0/8)
  - PR: faint Intensity in 1% nuclear reaction (Allred score 2/8)
  - Her2: strong Intensity in more than 10% membrane reaction (score 3+)
  - Ki67: 30% nuclear reaction

Prof. Amal Hareedy
Dr. Rana Fathy

قسم الباثولوجي

Pathology Report
[[DOCTYPE PATHOLOGY_REPORT]]

Patient name: Mervat Abd El Kadr Amen
Date: 4.1.2022

DIGITAL BILATERAL SOFT TISSUE MAMMOGRAPHY AND COMPLEMENTARY ULTRASOUND

Technique

- Bilateral MLO and CC views
- Bilateral 3D MLO DBT (Digital Breast tomosynthesis)
- Bilateral breast ultrasound

Report:

Follow up post NACTH showing:

- Left Breast:
  - Diffuse edema pattern with retro-areolar asymmetric density and nipple retraction.
  - Ultrasound shows diffuse edema pattern rendering prober assessment with retro-areolar altered parenchyma echogenicity and infiltrating the skin at 2 o'clock with skin ulceration measuring roughly 65x 40 mm compared to 57 x 54 mm previously.
  - Regressive course regarding the previously mentioned axillary lymph adenopathy currently largest seen at level I measures 13 x 23 mm with cortical thickness 10 mm.
  - No pathological enlarged supra clavicular lymph nodes.
- Right Breast:
  - Scattered fibro glandular density (ACR B)
  - No suspicious micro calcifications or architectural distortion detected.
  - Complementary ultrasound no cystic or solid mass lesions.
  - Normal skin thickness, and nipple-areola complex with average caliber of the retro-areolar central ducts.
  - Nonspecific axillary LN.

Opinion:

- left breast malignancy post NACTH showing mild regressive course (BIRADS 6).
- Normal sonomammography of right breast (BIRADS 1)

Best regards,
Dr. Aya Deabes,MSc.

Mammogram
[[DOCTYPE:MAMMOGRAM]]

Patient name: ميرفت عبد القادر أمين
Date: 26.07.2021

- No suspicious micro calcifications or architectural distortion detected.
- Complementary ultrasound no cystic or solid mass lesions.
- Normal skin thickness, and nipple-areola complex with average caliber of the retro-areolar central ducts.
- Enlarged axillary LN with diffuse cortical thickening and preserved fatty hila.

Opinion:

- left breast locally advanced malignant mass and extensive axillary nodal disease (BIRADS 5) for histopathological examination.
- Normal sonomammography of right breast (BIRADS 1)
- Right axillary enlarged LNs mostly reactive for FNAC confirmation

Best regards,
Dr. Amira H Radwan MD,
Breast Imaging consultant,
NCI, Cairo University

Mammogram
[[DOCTYPE:MAMMOGRAM]]

ROUSSY
INTERNATIONAL

Patient Name: Mervat Abdel Kader Ameen Emam	Date : 12/11/2025
Exam : CT CAP+C (CT Chest Abd. Pelvis)	Age : 49 Year(s)
Medical ID: 3191/2021	Pat. ID: 22383

POST CONTRAST MULTI-SLICE CT SCAN OF THE CHEST, ABDOMEN, AND PELVIS: -
Compared to the previous study dated 03/05/2025 the current study shows

I Chest:

- Clear left mastectomy operative bed. With few prominent ipsialteral axillary lymph nodes for sonographic correlation.
- **Progressive course** regarding the size of the previously noted left upper lung lobe small pulmonary nodule, currently measuring 10 x 9 mm compared to 6 mm .
- **Mild regressive course** of the left internal mammary and superior mediastinal lymph the largest is the former currently reaching 7 x3 mm compared to 12 x 10 mm .
- Stable multiple right axillary prominent lymph nodes.
- Left upper and lingular subpleural retciualtions, likely post radiation sequel.
- Still seen prominent other mediastinal lymph nodes some are calcified.
- No evidence of pleural effusion, thickening or masses seen.
- Normal cardiac size and shape with no gross abnormality of the cardiac chambers. No pericardial effusion detected.
- Normal CT appearance of the thoracic aorta and the great vessels.

II Abdomen & pelvis:

- The liver is of average size with homogenous CT texture. An ill defined segment IVb 15 x11 mm with faint enhancement .
- No intra-hepatic or extra-hepatic biliary radicles dilatation.
- Normal size & parenchymal texture of the spleen with no focal lesions seen.
- Normal size, parenchymal thickness & density of both kidneys with no stones or back-pressure changes.
- Normal appearance of the pancreas, aorta, I.V.C and both suprarenals.
- **Mild regressive** course regarding size of the previously noted right external iliac lymph nodes, the largest is currently reaching 10 x 7 mm comapred to 24 x 10 mm .
- Urinary bladder shows no definite stones or masses.
- Normal appearance of the other pelvic organs.
- No abdominal or pelvic masses or collections.

***Scanned skeleton:**

- Diffuse osteopenia.
- Still seen central wedging of D12 vertebra with mild retropulsion; otherwise no definite worrisome lytic or sclerotic focal lesions.

Opinion:

- **Progressive course regarding size of the previously noted left upper lung lobe small pulmonary nodule.**
- **Mild regressive course regarding the left internal mammary and superior mediastinal lymph node and external iliac L. Ns .**
- **Stationary rest of the study.**

Dr. Reham Khedr ,, M.D.

CT Scan
[[DOCTYPE:CT_SCAN]]

Kasr El-Ainy Center of Radiation Oncology and Nuclear Medicine
Faculty Of Medicine, Cairo University
"HEMROCK"

مركز قصر العيني
لعلاج الأورام والطب النووي
كلية الطب – جامعة القاهرة
(تهروك)

Patient Name:	Mervat abd alqader Amin Emam	Patient Gender:	Male
ID:	Q42217	Date:	9/9/2024 2:22:53 PM

Whole body ^{18}F-FDG PET/CT

Clinical History

- A patient with history of left breast cancer, for assessment.

Procedure:

- Whole body 18F-FDG images from skull vertex to mid thighs. CT images of the same level.

Injected dose: 245 MBq	Injection time: 10:15	Acquisition time: 11:15
BGL: 114 mg/dl	Weight: 88 Kg	Liver SUVmax : 3.7
IV contrast: no	Oral contrast: water	Time/bed position: 2min
First PET/CT study.		

Opinion

- **Metabolically active metastatic lymph nodal, pulmonary and left pleural deposits as described.**

PET Scan
[[DOCTYPE:PET_SCAN]]